



Testing mental health inpatients for alcohol and/or illicit substance use

Policy code	SMHS151
Original endorsement date	2003
Date of current endorsement	September 2023
Functional subgroup	Mental Health
Summary	The use of alcohol and/or illicit substances, and/or medications not prescribed by the treating team by a mental health inpatient of the South Metropolitan Health Services (SMHS) is not permitted. This document outlines the principles and processes for requesting a patient to participate in appropriate testing (i.e., Urine Drug Screen, breathalyser, blood test), if there is reasonable concern that an inpatient has consumed alcohol and/or an illicit substance.
Policy owner	Executive Director Clinical Service Planning and Population Health
Contact	Anureet Johl, 6152 1169
Review date	September 2026
NSMHS Standards	Standard 1: Rights and Responsibilities Standard 8: Governance, leadership, and management Standard 10: Delivery of Care
NSQHS Standards	Clinical Governance Comprehensive Care Communicating for Safety Recognising and responding to acute deterioration
Chief Psychiatrist's Standards for Clinical Care	Chief Psychiatrist's Standards for Clinical Care as required by the Mental Health Act 2014
Status	Approved

1. Background

1.1. About this document

The use of alcohol and/or illicit substances, and/or medications not prescribed by the treating team by a mental health inpatient of the South Metropolitan Health Services (SMHS) is not permitted. This document outlines the principles and processes for requesting a patient to participate in appropriate testing (i.e., Urine Drug Screen, breathalyser, blood test), if there is reasonable concern that an inpatient has consumed alcohol and/or an illicit substance.

1.2. Key Definitions

Term	Definition
Alcohol	Alcohol is a central nervous system depressant/sedative drug. Even in small doses alcohol significantly impedes motor and cognitive function depending on the dose and individual susceptibility.
Blood Test	Medical procedure in which a sample of blood is collected from a person's body for laboratory analysis.
Breathalyser	Device which can be used to measure the blood alcohol content in an individual's system; measures the concentration of alcohol in the lungs.
Illicit Substance	Any chemical substance used for its effects on bodily processes.
Intoxication	Condition resulting from use of a psychoactive substance/s—that produces behavioural and/or physical changes.
Urine Drug Screen (UDS)	A rapid, visual panel of immunoassays that can be used for the simultaneous qualitative detection of multiple drug metabolites in human urine. Widely accepted method for alcohol and other drug screening.

2. Policy Statement

Patients will be informed:

- on admission that the use of alcohol, illicit substances and/or medications not prescribed by the treating team is not permitted.
- that testing may be undertaken in situations where there is reason to suspect alcohol/illicit substance use and where such use could impact the patient's mental state or could compromise the patient's treatment plan.

All SMHS Mental Health services are to develop protocols and procedures as required.

[FSFHG Alcohol and Other Drugs: management of addiction, toxicity and withdrawal](#)

[FSFHG Non- Prescribed Substances: Management on Mental Health inpatient wards](#)

[RkPG Alcohol and Other Drugs: Management of addiction, toxicity and withdrawal](#)

2.1. Consent

Only patients who provide informed consent may be tested.

- Where there is a strong suspicion of alcohol / other drug use and/or signs of intoxication staff will discuss possible substance use with the patient. The discussion will include harm reduction strategies and the requirement for testing where use could impact the consumer's physical or mental state or compromise the patient's treatment plan and outcomes.
- The patient's consent must be obtained prior to testing for alcohol and/or illicit substance use and documented accordingly.
- The patient has the right to refuse to be tested.

Refer to:

- [WA Health Consent to Treatment Policy](#)

2.2. Patient refusal

If the patient refuses testing, their behaviour, demeanour and actions will be documented and discussed with the treating team/ Nurse Unit Manager. Outside of business hours, the afterhours duty medical Officer / HOOT Clinical Nurse Specialist should be consulted. In both instances, a medical examination should be conducted to rule out any organic causes of the patient's behaviour, demeanour, and actions.

A decision regarding future care may be made based on the patient's presentation if there is a strong likelihood of intoxication. Refer to the local protocols and procedures.

3. Roles and Responsibilities

3.1. Executive

Each SMHS site or service executive director will ensure that managers and supervisors are aware of the requirement to implement guidelines and safe work procedures relating to testing mental health inpatients for alcohol and/or illicit substance use.

3.2. Managers/supervisors

Managers and supervisors should:

- Undertake risk assessments of tasks involving testing mental health inpatients for alcohol and/or illicit substance use.
- Ensure the development of relevant protocols and procedures
- Ensure staff are trained in relevant safe work procedures and manual tasks
- Where available and appropriate, ensure appropriate equipment is provided/available for the testing mental health inpatients for alcohol and/or illicit substance use

3.3. Staff

Staff should:

- Comply with mandatory training requirements and proactively seek further training as required
- Comply with safe work procedures
- Ensure appropriate equipment is used
- Ensure breathalyser is in good working condition/regularly tested
- Ensure urine analysis kits have not expired before administering
- Ask for assistance if required and involve relevant medical staff in the decision-making process regarding testing a patient

4. Training

Staff administering tests to detect alcohol or illicit substances must have received adequate instruction in the correct administration of the test and result interpretation.

Only staff who have been trained in the use of the equipment are to conduct the testing, following the manufacturer's instructions, as per site requirements.

5. Equipment

Equipment for testing will be correctly stored and maintained.

SMHS Health Technology Management has the responsibility for maintenance of alcohol breathalysers used for testing of patients. The equipment must be registered on a data base maintained by SMHS Health Technology Management.

A certificate of correct calibration will be provided to the clinical unit by SMHS Health Technology Management.

6. Documentation

Documentation must be completed in the patient's digital medical record (DMR) when there is reasonable concern that an inpatient has consumed alcohol and/or illicit substance and/or medications not prescribed by the treating team.

The following must be fully documented:

- Circumstances leading to the request for testing.
- Consultation and discussion with medical staff.
- Explanation to the patient.
- Patient's consent (or refusal).
- Testing and results; and
- Review of the treatment plan, including any requirements for withdrawal management.

7. Incident Reporting

Clinical incidents related to alcohol/and or illicit substance use are to be notified using the WA Health Datix Clinical Incident Management System (CIMS).

Documentation for incident reporting must comply with Department of Health policy.

Refer to:

- [Department of Health Clinical Incident Management Policy \(2019\) MP 0122/19](#)

8. Compliance monitoring and legislative penalties

8.1. Compliance monitoring

Each SMHS site or service executive director is to ensure compliance with this policy.

8.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

9. Legislation

- *Mental Health Act 2014*

10. Additional references

10.1. Department of Health policies

- [Department of Health Clinical Incident Management Policy \(2019\) MP 0122/19](#)
- [MP 0062/17 Alcohol and Other Drug Withdrawal Management Policy](#)

11. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
MP V5	13/09/2023	08/09/2023	08/09/2026	08/09/2026	Original version

12. Approval

Approval by	SMHS AEG- Kate Gatti Executive Director Clinical Services
Approval date	08 September 2023

This document can be made available in alternative formats on request.

South Metropolitan Health Service

Locked Bag 100 Palmyra DC WA 6961

Telephone (08) 6152 2222

Email: Policy.SMHS@health.wa.gov.au

www.southmetropolitan.health.wa.gov.au

Compiled: South Metropolitan Health Service 2017

© State of Western Australia, South Metropolitan Health Service 2017